

1. Administrative & Study Identification

Full Study Title: A Phase 1/2 Study to Assess Safety, Pharmacokinetics, Immunogenicity and Preliminary Efficacy of BMS-986213 (Nivolumab-Relatlimab Fixed-dose Combination) in Chinese Participants With Advanced Solid Tumors **Short Title/ Acronym:** RELATIVITY 059 **Protocol Number:** CA224-059 **NCT Number:** NCT05134948 **Sponsor Name:** Bristol-Myers Squibb **Investigational Product:** BMS-986213 (nivolumab-relatlimab fixed-dose combination) **Phase of Development:** Phase 1 / Phase 2 **Medical Monitor:** Bristol-Myers Squibb Clinical Medical Director

2. Study Synopsis & Rationale

2.1 Study Objective Primary Objective: To assess the safety, drug levels (pharmacokinetics), immunogenicity, and preliminary efficacy of BMS-986213 (nivolumab-relatlimab fixed-dose combination) in Chinese participants with advanced solid tumors. **Secondary Objectives:** To evaluate progression-free survival (PFS), overall survival (OS), objective response rate (ORR), and duration of response (DOR) for the fixed-dose combination.

2.2 Background & Rationale To address the unmet medical need in patients with advanced solid tumors who have progressed on standard therapies. The simultaneous targeting of the LAG-3 and PD-1 immune checkpoints using a fixed-dose combination (nivolumab-relatlimab) has demonstrated superior efficacy compared to PD-1 blockade alone in certain malignancies, such as advanced melanoma. This study evaluates the specific safety, tolerability, and pharmacokinetic profile of this combination in a Chinese patient population.

3. Study Design & Methodology

3.1 Design Framework Study Type: Interventional **Control Type:** Single-arm **Blinding:** Open-label (None) **Randomization:** Non-Randomized (Sequential Assignment)

3.2 Planned Enrollment & Duration Target \$N\$: 24 (Actual Enrollment) **Number of Sites:** 2 locations (Beijing and Fujian, China) **Estimated Study Start:** 30-Nov-2021 **Estimated Primary Completion:** 01-Sep-2025 (Study Completion: 24-Oct-2025)

4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria 1. Presence of at least one lesion with measurable disease as defined by RECIST v1.1 criteria for response assessment. 2. Participants must have received, and then progressed, or been intolerant to at least one standard treatment regimen in the advanced or metastatic setting, if such a therapy exists. 3. ECOG performance status of 0 or 1. 4. Life expectancy of ≥ 12 weeks at the time of informed consent per Investigator assessment.

4.2 Exclusion Criteria 1. Participants with history of severe and/or life-threatening toxicity related to prior immune therapy (eg, anti-CTLA-4 or anti-PD-1/PD-L1 treatment or any other antibody or drug specifically targeting T-cell co-stimulation or immune checkpoint pathways). 2. Participants with an active, known or suspected autoimmune disease. 3. Participants with primary CNS tumors.

5. Intervention & Treatment Plan

5.1 Investigational Regimen Dosage/Frequency: BMS-986213 (nivolumab-relatlimab fixed-dose combination) administered as a regular IV infusion. **Route of Administration:** Intravenous (IV). **Duration of Treatment:** Continued until disease progression, unacceptable toxicity, or patient withdrawal.

5.2 Concomitant & Prohibited Therapy Permitted: Standard supportive care medications for disease-related symptoms. **Prohibited:** Concurrent systemic immunosuppressive therapies (e.g., high-dose corticosteroids) or other investigational anti-cancer agents.

6. Study Timeline & Visit Schedule

Visit ID	Window (Days)	Key Activities/Procedures	:---
:---	:---	Screening	Day -28 to 0 Informed Consent, Medical History, Tumor Imaging (RECIST v1.1), Baseline Labs
Baseline	Day 0	First Dose, Safety Assessment, Baseline PK sampling	Interim
	Specific Cycles	Safety Labs, Efficacy Assessment (Imaging), ADA Collection	End of Study
	Follow-up	Final Vital Signs, Exit Interview, Safety/Survival Follow-up	

7. Outcome Measures (Endpoints)

7.1 Primary Endpoint Definition: Incidence of Adverse Events (AEs), Serious Adverse Events (SAEs), safety lab abnormalities, and preliminary Objective Response Rate (ORR). **Measurement Method:** Clinical assessment using NCI CTCAE and central/local imaging review using RECIST v1.1.

7.2 Secondary & Exploratory Endpoints Progression-Free Survival (PFS) and Overall Survival (OS). Pharmacokinetic parameters (e.g., \$C_{max}\$, \$AUC\$) and Immunogenicity (Anti-Drug Antibody rates).

8. Safety & Adverse Event Reporting

Adverse Event (AE) Monitoring: Method of collection via routine clinical assessments and vital signs monitoring with a specific focus on Immune-Related Adverse Events (irAEs). **Serious Adverse Events (SAEs):** Reported within 24 hours of site awareness to the sponsor. **Data Monitoring Committee (DMC):** Internal safety monitoring managed by Bristol-Myers Squibb.

9. Statistical Analysis Plan (SAP) Summary

Analysis Populations: Safety Set (all participants who received at least one dose of the study drug) and Response Evaluable Population. **Sample Size Justification:** Target sample of \$N=24\$ provides sufficient statistical powering for preliminary observation of PK/PD, safety trends, and dose-limiting toxicities in Chinese patients. **Handling of Missing Data:** Missing objective tumor assessments are generally not imputed, rendering patients without post-baseline scans as non-responders for ORR.

10. Quality Assurance & Regulatory Compliance

GCP Compliance: This study will be conducted in accordance with Good Clinical Practice (GCP) and the Declaration of Helsinki. **Monitoring Plan:** Routine onsite and remote monitoring visits to ensure integrity of source data. **Audit Trail:** Validated eCRF tracking and data clarification mechanisms maintained by the sponsor.

11. Study Identifiers & Governance

Primary ID: CA224-059 **Registry Number:** NCT05134948 **Sponsor/Lead Organization:** Bristol-Myers Squibb **Study Title:** RELATIVITY 059 **Official Regulatory Title:** A Phase 1/2 Study of Relatlimab (Anti-LAG-3 Monoclonal Antibody) and Nivolumab (Anti-PD-1 Monoclonal Antibody) Fixed-dose Combination in Chinese Participants With Advanced Solid Tumors

12. Clinical Framework (Oncology Specific)

Primary Condition: Advanced Solid Tumors **Diagnosis Threshold:** Pathologically confirmed advanced/metastatic solid tumor with

measurable disease **Medical Methodology:** Dual Immune Checkpoint Blockade (anti-LAG-3 + anti-PD-1) **Optimization Target:** Maximum tumor volume reduction and progression-free survival prolongation

13. Study Procedures & Methodology

Management Pathway: Standard oncology treatment pathway for advanced malignancies **Education Protocol (HCP):** Training on identification and management of immune-related Adverse Events (irAEs) **Education Protocol (Patient):** Education on reporting immunotherapy-associated toxicity symptoms **Quality Audit Mechanism:** Source data verification and independent central radiological review

14. Observation & Follow-Up Schedule

Total Duration: Follow-up until death or study withdrawal **Onsite Visit Cadence:** Every 2-4 weeks during active treatment phase **Remote Monitoring Frequency:** Routine safety follow-up contacts **Checklist Review Interval:** Routine survival status checks every 12 weeks post-progression

15. Sign-off & Approval

Role	Name	Signature	Date	:--	:--	:--	:--
Principal Investigator	[Name]	_____	[DD-MMM-YYYY]				
Clinical Operations Lead	[Name]	_____	[DD-MMM-YYYY]		Lead		
Statistician	[Name]	_____	[DD-MMM-YYYY]				